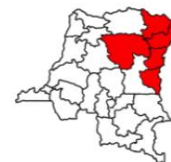




TASK FORCE PRESIDENTIAL EBOLA 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°097/MVEBDB/19/08/2026

Reporting date: August 19, 2026

Publication date: August 20, 2026



HIGHLIGHTS (last 24 hours)

As of August 19, 2026, 81 new confirmed cases of Ebola Virus Disease (EVD), including 23 community cases, were recorded in the provinces of Ituri (56 cases), North Kivu (18 cases), and Haut-Uélé (7). No new health zones have been affected.

ACCUMULATIVE CAS 5 290	 ACCUMULATIVE DEATH CONFIRMED 2 516	 LETHALITY 47,6%	PATIENT IN ISOLATION/CTE 837	ACCUMULATIVE HEALED 1 152	TRACKING RATES CONTACTS (Of the Day) 82,9 %
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uele, Ituri, North Kivu,
South Kivu, Tshopo & Lower Uele



56 out of 151
health zones affected
(37,1%)



Summary of key points from the last 24 hours

In the past 24 hours, **81 new confirmed cases**, including **40 community deaths** from Ebola Virus Disease (EVD), have been reported in the provinces of **Ituri (56 cases)**, **North Kivu (18)** and **Haut Uélé (7)**.

Over the past 24 hours, **37 patients** have been declared recovered after receiving two negative Ebola virus (EV) test results (36 in Ituri and 1 in North Kivu). On the other hand, **17 confirmed deaths within Ebola treatment centers** have been recorded, including 13 in Ituri and 4 in North Kivu.

The Democratic Republic of Congo has recorded **5,290 confirmed cases**, including **2,516 deaths**, representing a case fatality rate of **47.6 %**. Ituri remains the epicenter of the epidemic, accounting for 84.1% of cumulative confirmed cases and nearly two-thirds (69.1%) of new cases reported in the last 24 hours.

Currently, 56 of the 151 health zones in the six provinces have been affected since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (2/11).

Distribution of confirmed cases and deaths by affected province

(As of August 19, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	4447	1 984	44,6%	28/36 (77,8 %) 12/34	56
North Kivu	663	452	68,2%	(35,3 %) 6/13 (46,1	18
Haut-Uélé	160	71	44,4%	%) 7/23 (30,4 %)	7
Shop*	15	7	46,7%	1/34 (2,9 %)	0
South Kivu	3	1	33,3%	2/11(18,2%)	0
Bas Uélé	2	1	50,0%		0
Total	5 290	2 516	47,6%	56/151 (37,1 %)	81

*Harmonization of data with the province of Tshopo, bringing the cumulative number of confirmed cases from 14 to 15 (+1 retrospective case).

Confirmed cases and deaths by province and health zone (situation as of August 19, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4447	1984	44,6%	56	13	13	26
Give	11	1	9,1%				0
Underwood	7	6	85,7%				0
Ariwara	8	2	25,0%				0
Aungba	11	5	45,5%				0
Bamboo	114	26	22,8%				0
have	1	1	100,0%				0
Bunia	1244	389	31,3%	25	5		5
Ladies	25	9	36,0%				0
Drodro	13	6	46,2%				0
Fireworks	72	30	41,7%				0
Gethy	5	2	40,0%				0
Flounder	2	0	0,0%				0
Kilo	31	12	38,7%				0
Team	53	37	69,8%				0
Liter	186	106	57,0%				0
Logo	11	5	45,5%				0
Fight	17	4	23,5%				0
distributed	4	1	25,0%				0
Washing	14	7	50,0%				0
Darkness	25	16	64,0%				0
Mangala	183	105	57,4%	10	6		6
Mongbwalu	593	282	47,6%				0
Nanny	182	105	57,7%	4	1		1
Nizi	567	244	43,0%	9	1		1

Legumes	121	35	28.9%				0
Jungle	10	4	40.0%	1	0		0
Rwampara	886	332	37.5%	7	0		0
Tchomia	51	25	49.0%				0
A valves	THAT	187	THAT			13	13
North Kivu	663	452	68.2%	18	8	4	12
Me	104	76	73.1%	6	3	2	5
Butembo	127	98	77.2%	3	0		0
Ten	1	0	0.0%				0
Kalunguta	13	6	46.2%				0
Cut	316	210	66.5%	6	4	2	6
Kyondo	15	12	80.0%				0
Lubero	1	1	100.0%				0
Be careful.	2	1	50.0%				0
Masereka	7	4	57.1%				0
The museums	66	38	57.6%	3	1		1
Oicha	6	4	66.7%				0
Vuh	5	2	40.0%				0
Haut-Uélé	160	71	44.4%	7	2	0	2
Kill Mangbetu	23	9	39.1%				0
Gombari	1	1	100.0%				0
Mathematics	58	24	41.4%	3	0		0
Lining	20	14	70.0%				0
Beans	1	0	0.0%				0
Ordinary	57	23	40.4%	4	2		2
Boss	15	7	46.7%	0	0	0	0
Bafwasende	1	0	0.0%				0
Kabondo	3	1	33.3%				0
Lubunga	1	0	0.0%				0
Maquis-Kisangani	6	3	50.0%				0
Mangobo	2	2	100.0%				0
Boss	1	0	0.0%				
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	2	1	50.0%	0	0	0	0
Blind	1	1	100.0%				0
Viadana	1	0	0.0%				
Total	5290	2516	47.6%	81	23	17	40

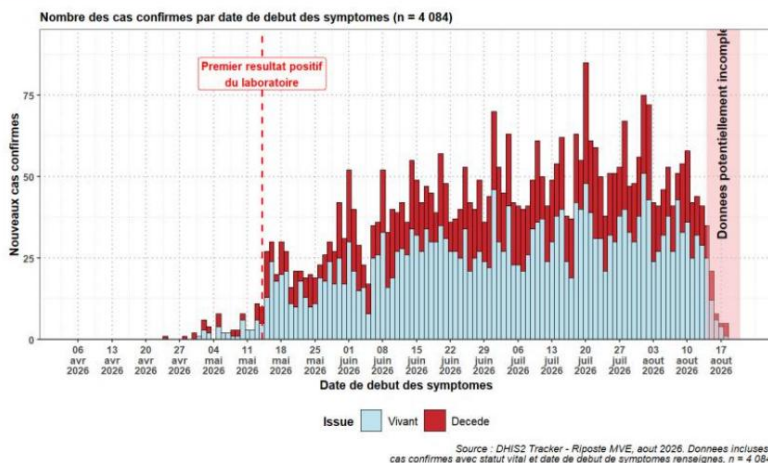
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA : Not Applicable.

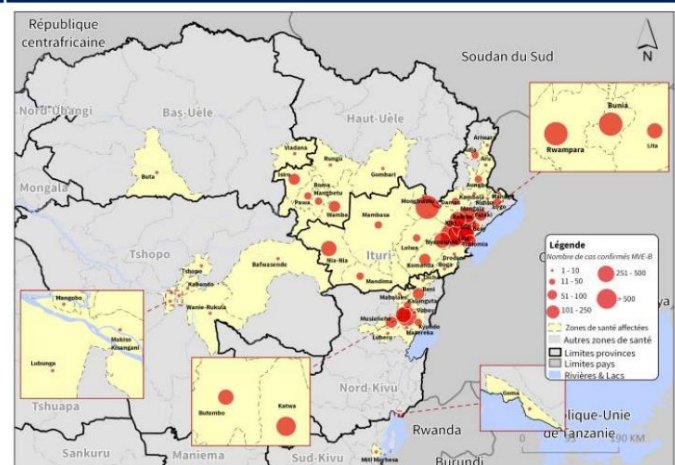
Status of alerts notified by province, as of August 19, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (as of today)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	ND	ND	0	ND	ND	ND	ND	ND	ND
Bas Uélé	6	0	6	3	0	3	0	3	3
Ituri	625	60	685	132	53	365	2	148	83
North Kivu	1 036	74	1110	91	71	932	0	160	88
South Kivu	6	0	6	3	0	0	0	3	3
Boss	97	0	97	13	0	84	0	13	8
Total General	1770	134	1 904	242	124	1384	2	327	185

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1.Coordination

1.1.1. Main Actions

- **Kinshasa:** A meeting was held, chaired by the Minister of Health, in the presence of technical and financial partners, dedicated to the review and validation of the second generation Ebola virus disease plan.
- Meeting at the office of the Minister of Health with His Excellency. The meeting focused on the presentation of the 180-day multisectoral preparedness and response plan for Ebola Virus Disease (EVD).
- **North Kivu :** continued preparation of the compassionate vaccination campaign and meetings of the sub-coordinations of Beni, Butembo and Katwa regarding the capacity to receive the CTEs of Kitumba and Katwa.

- **Ituri:** continued evaluation of the technical platform of the CTEs using the 5S tool, 9 CTEs having completed their encoding.
- **Tshopo :** launch of the compassionate vaccination process against Ebola, 2,000 doses of ERVEBO available in the country, deployed as a priority in the health zones of Makiso-Kisangani, Kabondo, Mangobo (Tshopo) and Buta (Bas-Uélé).

1.2. Epidemiological surveillance

1.2.1. Main Actions

At the end of the day on August 19, 2026, 1,904 alerts were recorded, including 1,752 (92.0 % were verified in the five reporting provinces. Of these alerts, 366 (23.8%) were validated as suspected cases and 327 (89.3%) were investigated, of which 185 were transferred to the CT/CTE.

The proportion of contacts monitored in the last 24 hours has risen to 82.9% (16 355 views out of 19,720 to follow), although it remains below the 85% threshold: Ituri 87.6 % (9,204/10,509), Tshopo 100.0% (473/473) and North Kivu 80.5% (5,830/7,540). Haut-Uélé 56.6% (848/1,498) and Bas-Uélé 59.5% (131/220).

1.2.2. Challenges

Poor contact tracing around confirmed cases, mainly in the provinces of Haut Uélé and Bas Uélé linked to an inadequacy of response teams.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

- Two (2) alerts were notified to the PoE/PoC in the last 24 hours, both in North Kivu, neither of which was validated after investigation; Ituri, South Kivu, and Tshopo did not notify any alerts, and Haut-Uélé did not report any (ND). No bodies or problematic contacts were intercepted at the points of entry.

Monitoring of indicators at PoE/PoC by province, as of August 19, 2026

Indicators	Ituri	North Kivu	South Kivu	Global
Proportion of PoC enabled and PoE enhanced	98,3 %	100 %	100 %	98,7 %
Number of people who have switched to PoE/ PoC	148 032	80 917	84 432	313 381
% of travelers screened % of	98,0 %	97,2 %	100 %	98,3 %
travelers who washed their hands	98,0 %	ND	100 %	98,7 %
% of travelers made aware	98,0 %	ND	100 %	98,7 %
Number of alerts notified	0	2	0	2
Number of validated alerts alive	0	0	0	0
Number of bodies recovered today	0	0	0	0
Number of problematic contacts intercepted today	0	0	0	0
Percentage of validated live alerts referred to the CT/CTE	THAT	THAT	THAT	THAT
% of lifeless bodies swabed	THAT	THAT	THAT	THAT

1.3.2. Challenges

- Insufficient operationalization of PoE/PoC: in Tshopo, only 17 points of entry
Of the 22 planned PoCs activated (35% of PoEs activated and reinforced), only 57.1% of reports are complete; in Ituri, the completeness rate is 98.3% (59 out of 60 reports). Haut-Uélé and Tshopo did not submit any data.
- Constraints affecting the functionality of PoE/PoC: non-payment of service providers in North Kivu (only 5 out of 18 paid) and in Ituri, where dissatisfaction also relates to the per diem rate; persistent inconsistencies between PoE/PoC alert data and those of health areas in DHIS2 Tracker; absence of isolation device in the PoCs of Tshopo; low screening where men in uniform are not involved.

1.4. Laboratory

1.4.1. Main actions

- **Ituri : 56 new positive results** (43 living and 13 dead) out of 265 new samples received and analyzed (**positivity: 21.1 %**).
- **North Kivu : 18 new positive results** (10 living and 8 dead) out of 127 samples analyzed (**positivity of 15.0 %**).
- **Haut-Uélé: 7 new positive results** (5 living and 2 dead) out of 30 samples analyzed (**positivity of 23.3%**).

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- **In Ituri**, 19 of the 70 planned infection control rings were opened (27.1%), and 40 of the 82 identified accommodation sites were decontaminated (48.8%); 7 IPC kits were distributed, 8 assessments and 24 ESS support sessions were conducted, and 106 service providers were briefed; of 82 death alerts received, 62 bodies were collected by swab, 13 were unsuccessful, and 77 EDS (Emergency Diagnosis System) procedures were carried out.
- **In North Kivu**, 9 health facilities and 5 households were decontaminated, and 5 health establishments were assessed; 243 health facilities received support, and 799 service providers were briefed, with completion reaching 7 out of 12 health zones. Of 49 death alerts, 46 bodies were collected by swab (3 failed due to community resistance in the Kasanga health area, Beni health zone), and 47 blood tests were conducted, with 8 swabs returning positive results.
- **In Bas-Uélé**, two rooms at the Buta General Referral Hospital (laboratory and treatment room) were decontaminated, and 8 out of 13 ESS (Emergency Health Services) received support; decontamination of two households could not be carried out due to family resistance. Regarding one death alert received at the Likati General Referral Hospital, no body was recovered, nor was a DHS (Digital Health Service) performed, due to family opposition.
- **In Tshopo**, one ring was opened around the 15th case (Makiso-Kisangani) with decontamination of the household and the social and solidarity economy (SSE); 14 SSE support visits, 22 briefings for **service providers**, and 4 triage unit installations were carried out. Of the 2 death alerts received (Tshopo Health Zone), 2 bodies were swabbed.

1.5.2. Challenges

- Weak implementation of IPC standards: limited ring openings in Ituri (19 out of 70) and incomplete decontamination (40 out of 82 sites), due to insufficient decontamination teams, missing addresses, and insecurity in some villages; resistance to EDS in Ituri (Aru, Bambu, Rwampara, Nia-Nia, Tchomia, and Mandima) and

Stone-throwing of an EDS vehicle in Komanda; family resistance in Bas-Uélé preventing the decontamination of two households and the EDS of a suspicious death at the HGR in Likati; very low PCI scores in several ESS in Tshopo (11.6% at the William Clinic, 12.6% at the Papa Freddy Health Center and 12.9% at the Military Pavilion).

1.6. Continuity of care

1.6.1. Main actions

- **In Ituri**, 138 new admissions and 6 readmissions were recorded, with 129 discharges, including 36 recoveries. Thus, 551 patients are hospitalized in 965 beds, representing an overall occupancy rate of 57.1%. Twenty-seven out of 30 ambulances are available, with 14 in use and 25 successful missions.
- **In North Kivu**, 46 admissions were recorded (4 confirmed, including 2 at the Katwa Ebola Treatment Center and 2 at the Beni Treatment Center, and 42 suspected cases) with 55 discharges, including 1 recovery at the Vuhovi Treatment Center. Thus, 196 Patients are hospitalized in 206 beds, representing 95.1% occupancy.
- **In Haut-Uélé**, 53 patients are being treated (36 confirmed and 17 suspected); 52 beds from the Wamba and Isiro General Referral Hospitals have been allocated to the Ebola Treatment Centers and a pavilion from the Wamba General Referral Hospital has been fitted out, bringing the provincial capacity to 130 beds (Occupancy rate 44.6%).
- **In Haut-Uélé**, 4 patients are in hospital (Occupancy rate 100.0%).
- **In Tshopo**, a new admission of a suspect to the CTE HC Kisangani brings to 5 the number of patients isolated for 14 beds (41.7% occupancy).
- **In South Kivu**, 25 suspected cases remain hospitalized.

1.6.2. Challenges

Insufficient capacity and technical facilities at several Ebola Treatment Centers (ETCs): the ETCs in Bambu (confirmed and suspected cases) and Nizi, Kilo, Logo, and Kigonze (suspected cases) are at capacity in Ituri, where 77 patients (Kilo 13, Sota 16, Bambu 38, Lolwa 9, and Mangala 1) are not receiving any food assistance; in North Kivu, bed occupancy for suspected cases has reached 103%, and several suspected cases in Butembo, Masereka, Vuhovi, and Kalunguta are isolated in health centers that are not transit centers; in Tshopo, the ETC has low capacity, experiences power outages, lacks safe blood, and has insufficient biological equipment; in Bas-Uélé, no ambulances are available for the response. Three patients have escaped in Ituri (1 in Kigonze and 2 in Nizi).

1.7. Risk communication and community engagement

1.7.1. Main actions

- **In Ituri**, 543 new contacts were listed around the day's cases and 235 people were reached through group and individual interviews in seven health zones (Rwampara, Mangala, Bunia, Nia-Nia, Nizi, Bambu-mine and Mongbwalu); 3,244 people were reached during home visits and 5,252 during mass awareness campaigns.
- **In Ituri**, the SMSPS pillar covered 31 of the 53 new confirmed cases (58.5%) in 4 of the 13 health zones that reported cases, 58 new suspected cases, and 15 of the 36 recovered patients reintegrated into the community (41.7%); 70 affected households received support, and 50 separated or orphaned children received community follow-up. In Tshopo, two areas of resistance were overcome in Mangobo and Makiso, 107 people received psychoeducation, and 1,341 people were reached through home visits in Bondeko, Mangobo, Tshopo, and Lubunga.

- **In North Kivu**, mass communication and engagement of 90 community leaders from three health areas (Kuka, Ngadi and Boikene) at the Beni urban command headquarters regarding adherence to barrier measures and community involvement in the fight against Ebola; awareness-raising of 6 people to facilitate the decontamination of 3 households in Vuhovi and listing of 37 contacts of the confirmed case of Ebola in Ngongolio (Beni health zone); 26 cases were covered and 60 household heads were affected.

1.1.1. Challenges

- Persistent community challenges: violent resistance to EDS in the village of Mungamba (Komanda Health Zone, Ituri), where a vehicle was damaged and the team escorted by the FARDC; two refusals of EDS for religious reasons in Bambu (Peti Health Zone), one of which remains unresolved; attempted arson of the houses of two RECOs in the Mangothe Health Zone (Beni Health Zone) and assault of a member of the EDS team in the Kasanga Health Zone; claim of non-payment of the zonal CREC, IT, CODESA and RECO teams in Nyankunde; low coverage of SMSPS interventions around the latest confirmed cases in Bunia (13 of the 25 notified cases).

1.8. Logistics

1.8.1. Main actions

In Ituri, 16 vehicles donated by UNICEF were received for decontamination teams in six health zones and 856 kg of PECH kits (118 packages) delivered to the Lolwa health zone, with IPC kits in Rwampara and PECH kits in the Butembo and Katwa health zones; 250 liters of fuel were made available to ambulances and response vehicles.

In Tshopo, a delivery bed was provided to the Madula Health Center (PK 23 RN4) and thermoflash devices were delivered to the health zones of Makiso-Kisangani and Tshopo; construction work on the isolation units in Madula and Kabondo and the large-capacity CTE continues.

In North Kivu, IPC supplies donated by FHI 360 (gloves, sprayers, disinfectants, sachets, and Tyvek suits) were received and delivered to EDS teams in seven health zones; a tent was delivered to the response coordination team, and nutritional supplies were provided in Oicha. Construction of the triage area at Vuhovi General Referral Hospital (ULB Cooperation) and the rehabilitation of the treatment center at Musienene General Referral Hospital (MSF France) are ongoing.

In Bas-Uélé, the pillar packaged the PCI inputs destined for the Viadana health zone and ensured the transport of samples from the health zones to the Rad-One site.

1.8.2. Challenges

- 1.9.** Persistent logistical needs to support the expansion of the response: completion of the extensions of the CTEs of Nizi, ISTM and Bambu in Ituri and needs for 4 glove boxes (ZS of Fataki, Bambu-mine, Lita and Rwampara), cryotubes and cryoboxes, three ambulances being in disrepair (1 in Bambu and 2 in Nizi); deployment of IPC inputs to the 7 zones of the west and north of Bas-Uélé, where hydroalcoholic gel and examination gloves are on the verge of running out and where the lack of Starlink kits limits communication to uncovered health areas; in Tshopo, mobility deficit (22 motorcycles available out of 200 required) and shortages of inputs.

1.10. Security

1.10.1. Main actions

- In North Kivu, an EDS team was attacked in the Bunji health district, in Kasanga Tuha (several injured and one probable death), the team having left without requesting security escort; the pillar provided six security coverages for EDS and decontamination teams in Beni and Katwa, restored public order at the Ndindi health center which was threatened with fire and updated the mapping of recommended accommodation sites for Beni and

Butembo. In Ituri, an EDS vehicle was stoned in the village of Mungamba (Komanda Health Zone). Challenge: Ituri, Haut-Uélé, Tshopo, and Bas-Uélé have not submitted consolidated security data.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions

- In Ituri, 77 people received PSEA briefings (30 participants, including 12 women, in Bunia and 47, including 20 women, in Rwampara), and all signed the code of conduct; 210 people were sensitized in the community (157 in Bunia, 15 in Rwampara, and 38 pregnant women in Gety). Coverage remains limited to 4 of the 36 health zones (11.1%), and only two partners (WHO and Africa CDC) report their PSEA activities.

1.1.2. The challenges

Coverage of PSEA activities is still insufficient, limited to 4 health zones out of 36 in Ituri, lack of mapping of stakeholders, insufficient mass awareness materials and lack of means of transport and computer equipment for the pillar.

MESSAGES CLES

- Ebola virus transmission remains active, with 81 new cases including 23 community deaths and 17 intra-CTE recorded in the last 24 hours, bringing the national total to 5,290 confirmed cases and 2,516 deaths.
- Ituri remains the epicenter of the epidemic, concentrating 84.1% of cumulative confirmed cases and 69.1% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- Surveillance shows 89.3% of suspected cases investigated while the follow-up rate (82.9%) remains below the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of 64 support teams.



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